A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Pauline Tyndall
Cobram Medical Clinic

Dear Dr Tyndall,

Re: Ms Jessica Hansen
DOB: 07/08/1977 Phone: 0418 646 194
Address: 551 Ross Road, KATUNGA, VIC, 3640
Email: jessmjohns551@gmail.com

Summary

- **Stable gastrointestinal symptoms with no recent obstructive episodes.**
- **Iron deficiency and zinc deficiency secondary to coeliac disease and ileoanal pouch malabsorption.**
- **Mildly elevated prolactin with associated visual disturbances.**
- **For gastroscopy, pouchoscopy, and iron infusion under the same sedation.**
- **No action required by you**

I had the pleasure of reviewing Jessica Hansen. From a gastrointestinal perspective, she remains stable and has not experienced any further obstructive episodes since returning to Australia. We discussed her coeliac disease; she is currently approximately 95% gluten-free. I have reiterated the long-term risks of untreated coeliac disease, including small bowel lymphoma, though she acknowledges this is her choice. Recent pathology shows a decline in several nutritional markers. Her iron levels are lower than previously recorded, and while she is not yet anaemic, she is significantly iron deficient. This is likely a combination of menstrual losses and malabsorption related to both her coeliac disease and her prior pouch surgery. Her zinc is also low. Furthermore, her vitamin D is low, which is particularly concerning given her history of osteoporosis. Her B12 is currently at the low-normal range. Regarding her biochemistry, there is a mild elevation in prolactin. Jessica has reported some headaches and visual disturbances. While the elevation is slight, I have advised her to see an optometrist for a formal assessment. If the prolactin remains elevated or increases on repeat testing, I will arrange an MRI of her brain to evaluate the pituitary gland.

Impression and Plan

I have recommended that Jessica undergo a gastroscopy and a pouchoscopy (flexible sigmoidoscopy of the pouch). To address her iron deficiency, I have suggested we perform an iron infusion in the recovery room immediately following her procedures. I have advised that an enema be administered prior to the pouchoscopy to ensure adequate visualisation of the mucosa. In the interim, she should commence a zinc supplement and high-dose vitamin D. She may also require B12 injections, which she may see you for. I have encouraged her to see an optometrist regarding her vision, and I will monitor her prolactin levels closely. We will coordinate her procedures at St Vincent's or a similar private facility at a time convenient for her.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Mr Michael Johnston

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002

Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037

St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065